

HCG — Subcutaneous Injection Instructions

PROVISIONAL CLINICAL CONTENT — This protocol has been drafted using current evidence-based parameters and is operationally usable upon Provisional approval. Full clinical review and final approval by the Covering Physician are scheduled within 180 days. If you have any questions about the technique described in this document, contact your Bloom Metabolics care team via the patient portal before injecting.

Protocol ID: Protocol-003

Medication: Human Chorionic Gonadotropin (HCG)

Route: Subcutaneous (SubQ)

Version: 0.1

Effective Date: TBD (pending approval)

Reviewed by: Pending Covering Physician Review

Before You Begin

Gather the following supplies before starting:

- & HCG vial (check expiration date and reconstitution date — do not use if expired or past the use-by window)
- & Insulin syringe: 29-gauge to 31-gauge, 5/16" (8mm), 0.5mL or 1mL barrel
- & Alcohol swabs (2 — one for the vial top, one for the injection site)
- & Sterile gauze pad
- & FDA-cleared sharps container
- & Clean, flat surface with good lighting

If your HCG arrives as a powder that requires reconstitution, see the Reconstitution section below before proceeding.

Before every injection:

1. Wash your hands thoroughly with soap and water for at least 20 seconds
2. Inspect the vial — the reconstituted solution should be clear and colorless. If the solution is cloudy, discolored, or contains particles, do not use it. Contact your care team.
3. Check the expiration date on the vial AND the date you reconstituted the medication (write the reconstitution date on the vial label)
4. If the medication has been stored in the refrigerator, it may be injected cold or allowed to reach room temperature for a few minutes — either is acceptable

If something does not look right — the vial is damaged, the solution looks unusual, or the syringe packaging is open — do not inject. Contact your care team.

Reconstitution (if applicable)

If your HCG is supplied as a lyophilized (freeze-dried) powder with a separate vial of bacteriostatic water, you must reconstitute it before use:

1. Clean the tops of both vials (HCG powder and bacteriostatic water) with alcohol swabs
2. Using a syringe, draw the specified volume of bacteriostatic water as instructed by your compounding pharmacy
3. Inject the bacteriostatic water slowly into the HCG powder vial, directing the stream against the glass wall (not directly onto the powder)
4. Gently swirl the vial until the powder is fully dissolved. Do not shake — shaking can damage the protein structure
5. The solution should be clear and colorless after reconstitution. If it is cloudy or contains particles, do not use it.
6. Write the reconstitution date on the vial label
7. Store the reconstituted vial in the refrigerator immediately
8. Use within the timeframe specified by your compounding pharmacy (typically 30–60 days post-reconstitution)

Identifying Your Syringe

This protocol uses an insulin syringe. This is a small syringe with a fixed (non-removable), very thin needle.

Feature	Insulin Syringe (USE THIS)	Intramuscular Syringe (DO NOT USE)
Barrel size	0.5mL or 1mL	1mL or 3mL
Needle	Fixed (non-removable), 29g–31g, short (5/16")	Detachable, 23g–25g, longer (1 inch)
Needle thickness	Very thin and flexible	Thicker and sturdier
Typical use	Subcutaneous injections (into fat tissue)	Intramuscular injections (into muscle)

If your shipment contains intramuscular syringes (the longer ones), do not use them for HCG. Contact your care team immediately.

Injection Sites

Recommended Sites

Abdomen (Preferred)

The fatty tissue of the lower abdomen, at least 2 inches from the midline on both right sides. Avoid scars, bruises, and stretch marks. Do not use for subcutaneous HCG.

How to locate: Place two fingers on either side of your navel, about 2 inches apart. The area between your fingers and extends outward into the lower abdomen is the recommended site.

Anterior Thigh

The front of the thigh, in the middle third between the hip and knee, where subcutaneous fat is accessible.

How to locate: Divide your thigh into thirds (top, middle, and bottom). The middle third is the recommended site.

Site Rotation Schedule

Rotate injection sites with each administration. Alternate between left and right sides. Vary the exact location by at least 1 inch from the previous injection to prevent lipohypertrophy.

Example 4-week rotation:

Week	Site	Side	Date	Notes
1	Abdomen	Right	_____	_____
2	Abdomen	Left	_____	_____
3	Anterior Thigh	Right	_____	_____
4	Anterior Thigh	Left	_____	_____

Fill in the date and any observations a

Step-by-Step Injection Technique

Drawing Medication

1. Clean the top of the HCG vial with an alcohol swab. Allow it to air dry.
2. Remove the insulin syringe from its packaging. Do not touch the needle.
3. Pull back the plunger to draw air into the syringe equal to the volume of your prescribed dose.
4. Insert the needle into the vial through the rubber stopper.
5. Inject the air into the vial.
6. Invert the vial and slowly pull back the plunger to draw your prescribed dose into the syringe.
7. Check for air bubbles. If present, tap the syringe gently with your finger to move bubbles to the top, then push the plunger slightly to expel the air. Re-check the volume.
8. Remove the needle from the vial.

Injecting

1. Clean the injection site with a fresh alcohol swab using a circular motion from center outward. Allow to air dry completely.
2. Pinch a fold of skin at the injection site with your non-dominant hand. Hold the pinch throughout the injection.
3. Hold the syringe like a pen or dart with your dominant hand.
4. Insert the needle at a 90-degree angle into the pinched skin fold in one smooth, quick motion. Insert the needle to its full depth.
5. Inject the medication slowly and steadily over 5–10 seconds.
6. After injecting the full dose, wait 5 seconds before withdrawing the needle.
7. Withdraw the needle in one smooth motion.
8. Release the skin pinch.
9. Apply gentle pressure with a sterile gauze pad. Do not rub the injection site.

After Injection

1. Immediately dispose of the used syringe in your sharps container. Do not recap the needle.
2. Record the injection in your rotation tracker (site, side, date, and any observations).

What to Expect After Injection

Normal findings (not cause for concern):

- A small drop of blood at the injection site
- Slight redness or mild itching at the injection site that resolves within hours
- A small, firm lump under the skin that resolves within a few days

Abnormal findings (contact your care team):

- Injection site redness, swelling, or pain that worsens or persists beyond 3 days
 - Drainage or pus from the injection site
 - Fever following injection
 - Allergic reaction symptoms (hives, swelling of face/lips/tongue, difficulty breathing)
 - Unusual headache, mood changes, or swelling that was not present before starting HCG
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Troubleshooting

I saw blood after removing the needle.

A small drop of blood at the injection site is normal. Press gently with a clean gauze pad. This does not affect the medication.

The needle bent during injection.

Do not attempt to straighten or reinsert a bent needle. Discard the entire syringe in your sharps container and prepare a new dose with a fresh syringe.

I missed my scheduled injection day.

Administer your injection as soon as you remember. If it is very close to your next scheduled dose, contact your care team for guidance. Do not double your dose.

The injection site is red, swollen, or painful 24+ hours later.

Mild site reactions are common and usually resolve within 1–2 days. If the area becomes increasingly red, warm, or swollen, or if you develop fever, contact your care team.

I am not sure I injected the full dose.

Check that the plunger is fully depressed. If so, you received the full dose. Do not attempt to re-inject. Note this at your next check-in.

How do I reconstitute my HCG?

See the Reconstitution section above. Follow the instructions provided by your compounding pharmacy. If you are unsure about the correct volume of bacteriostatic water or the reconstitution process, contact your care team before reconstituting.

When to Contact Your Care Team

Contact your Bloom Metabolics care team via the OptiMantra patient portal if you experience any of the following:

- Injection site signs of infection (increasing redness, warmth, swelling, drainage, fever)
- Allergic reaction symptoms
- Unusual symptoms not present before starting HCG
- Questions about reconstitution
- Uncertainty about your technique — contact us BEFORE injecting
- Any situation not covered by this troubleshooting guide

Emergency: If you are experiencing a medical emergency — difficulty breathing, severe allergic reaction, loss of consciousness — call 911 immediately. Do not delay emergency care to contact your clinic.

Storage and Disposal

Medication Storage

- Refrigerate after reconstitution at 36–46°F (2–8°C)
- Do not freeze — frozen HCG must be discarded
- Use within the timeframe specified by your compounding pharmacy (typically 30–60 days post-reconstitution; check the label)
- Protect from light
- Unreconstituted HCG powder may be stored at room temperature per manufacturer guidance — check the label
- Do not use if the solution is cloudy, discolored, or contains visible particles
- Do not use past the expiration date or beyond the reconstitution use-by window

Sharps Disposal

- Dispose of all used syringes immediately in an FDA-cleared sharps container
- Do not recap needles
- When the sharps container is three-quarters full, seal it and follow your local community disposal guidelines

Unused Medication

- Do not flush medication down the toilet
 - Return unused or expired medication to your pharmacy or follow local disposal guidelines
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Acknowledgment

Before your first self-injection, you will acknowledge receipt and understanding of these instructions in the OptiMantra patient portal. The acknowledgment statement reads:

"I have reviewed the injection instructions for HCG. I understand the technique, the injection sites, the rotation schedule, and the signs of complications. I will contact my Bloom Metabolics care team via the patient portal with any questions before injecting."

If you have questions about any part of these instructions, contact your care team BEFORE completing the acknowledgment and BEFORE injecting.

Clinical References

- FDA Prescribing Information: Human Chorionic Gonadotropin. Current FDA labeling at time of protocol drafting.
 - AUA Clinical Guideline: Evaluation and Management of Testosterone Deficiency (2018, updated 2024) — HCG co-administration section.
 - Endocrine Society Clinical Practice Guidelines — HCG in the context of testosterone replacement therapy.
 - CDC Guidelines: Injection Practices — Subcutaneous Injections (current).
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This protocol is governed by SOP-004 (Patient Self-Administration Injection Education).

Protocol-003 · Version 0.1 · Generated 2026-05-03

